

CONSULTATION & COLLABORATION

Split-Treatment Collaboration Agreement

For a psychiatrist (or other prescriber) and a psychologist sharing a patient's care: roles, communication triggers, emergencies, releases and a patient-facing summary.

AUDIENCE

Psychiatrists · Psychologists

FORMAT

Agreement + patient page + log

VERSION

1.0 · Sept 2026

TIME TO ADAPT

30 minutes

WHAT IT IS

A written agreement between a prescribing clinician and a psychotherapist who share a patient's care, with a plain-language page for the patient and a communication log.

USE IT WHEN

A patient sees one clinician for medication and another for psychotherapy, and you want roles, communication and emergencies defined in advance.

CUSTOMIZE FIRST

Communication triggers and timelines, who provides after-hours coverage, each clinician's liability limits, and your state's consent and disclosure rules.

Why this matters

Split treatment is now the standard model for patients who need both medication and therapy, and it is also a documented risk. In a published audit of split-treatment coordination, only 36% of psychiatrists fully met coordination standards, and a missing signed release accounted for 87% of the cases with no coordination. Fragmented communication is most dangerous when a patient is suicidal, using substances, or changing medications. Risk-management guidance for psychiatrists recommends a written practice agreement with the collaborating therapist that covers emergency coverage and periodic record review.

THE THREE QUESTIONS TO SETTLE BEFORE YOU SHARE A PATIENT

1) Who holds each decision: diagnosis, medication, level of care, hospitalization. 2) What triggers an immediate call, and by which route. 3) Who covers each of you when you are away, and how the other finds out.

Part A: Collaboration agreement

Prescriber: name, credential, license and DEA state(s)

Phone / secure contact

Psychotherapist: name, credential, license and state(s)

Phone / secure contact

Patient name or ID

Date patient consented

Start date

1. Roles

	Prescriber	Psychotherapist
Core responsibility	Medical evaluation and differential diagnosis; medication decisions; monitoring of side effects, labs and interactions; controlled-substance stewardship	Psychotherapy; ongoing assessment of symptoms, functioning, safety and adherence; skills-building; psychoeducation
Diagnosis	Shares diagnostic formulation; owns medical differentials	Shares diagnostic formulation and observed course
Risk	Reassesses at each visit; owns medication-related risk (means, overdose potential)	Reassesses at each session; maintains the safety plan
Level of care	Either may recommend a higher level of care; decisions are discussed and documented	Same
Records	Keeps own record and shares what is needed	Keeps own record; psychotherapy notes as defined by HIPAA are not shared

Each clinician is responsible for their own clinical judgment and cannot delegate essential medical or psychotherapeutic care to the other. The therapist is not expected to monitor pharmacotherapy (for example labs or side-effect checks), but will report what they observe. The prescriber is not expected to conduct psychotherapy.

2. Patient consent and releases

Before information is exchanged, the patient signs an authorization allowing communication between the two clinicians (see the Privacy Notice and Release pack). HIPAA permits disclosures for treatment without authorization, but state law and Part 2 (for substance use disorder treatment records) may require more, and a signed release is the most common missing step in split treatment. If the patient refuses to allow communication between the clinicians, the two will discuss whether safe shared care is possible.

3. Routine communication

- Initial exchange within [7] days of the first shared visit: diagnosis, treatment plan, medications, risk and safety plan.
- Updates at least every [90] days, or every [1] visits, using the log at the end of this document.
- Medication changes: the prescriber tells the therapist within [1] business day. Therapy changes affecting risk or engagement: the therapist tells the prescriber within [1] business day.
- Preferred route: [secure message / phone]. Response time: within [2] business days for routine items.

4. Triggers for same-day contact

- ☐ New or worsening suicidal ideation, plan, intent or behavior, or an attempt.
- ☐ Psychiatric hospitalization, ER visit, or involvement of crisis or police.
- ☐ Serious violence concerns or threats toward others.
- ☐ Missed appointments in a row with the other clinician (more than [2]) or refusal to attend.
- ☐ Non-adherence to medication, abrupt discontinuation, or stockpiling.
- ☐ New substance use or a relapse, and any concern about misuse or diversion of prescribed medication.

- ☐ New medical diagnosis, pregnancy or plan to become pregnant, or breastfeeding.
- ☐ A plan to end treatment with either clinician, or a change in insurance or address affecting care.

5. Emergencies and coverage

For an emergency during business hours, the clinician who learns of it calls the other immediately. After hours, the patient is directed to [after-hours route] and 911 or 988. Each clinician provides the other with their coverage arrangements and tells the other in advance when they will be away. Prescription requests while the prescriber is away go to [covering prescriber].

6. Disagreements

If the clinicians disagree about diagnosis or treatment, they talk within [7] days. The patient's safety comes first. If disagreement continues, they seek a third opinion. If safe shared care is not possible, either may end the arrangement with reasonable notice and help the patient transition, so the patient is not left without care.

7. Liability and insurance

Each clinician maintains professional liability insurance with limits of at least [\$ per claim / \$ aggregate] and confirms with their carrier that split-treatment arrangements are covered. Each is responsible for their own acts. This agreement does not create a partnership, supervision or employment relationship. If either clinician is supervising or being supervised by the other, that relationship is documented separately.

8. Term

This agreement continues while both clinicians treat the patient, is reviewed every [12] months, and ends when either clinician's involvement ends.

Prescriber signature

Date

Psychotherapist signature

Date

Part B: Your care team (patient acknowledgment)

Give this page to the patient in plain language.

You are receiving care from two clinicians who will work together. **[Prescriber name]** manages your medication. **[Psychotherapist name]** provides your therapy. With your permission, they will share information that helps them care for you, and they will not share more than they need.

Who to contact for what. Medication questions, refills and side effects: **[Prescriber contact]**. Therapy, scheduling and how you are feeling: **[Psychotherapist contact]**. If something is urgent or you are unsure, contact either of them, and they will connect with each other.

Please tell both of us if you have thoughts of harming yourself, if you stop or change your medication, if you use alcohol or other substances, if you are pregnant or planning to be, or if you go to an emergency room.

In an emergency call 911. For a mental health crisis call or text 988, or chat at 988lifeline.org.

Patient signature

Date

Part C: Communication log

Date	From	To	Route	Topic (risk, medication, plan, other)	Action / follow-up

IN PSYALLIANCE

Partners in your Network are a natural pool for split-treatment collaborators. Filter by specialism, state and whether they accept referrals, then use in-context messaging to agree terms.

Sources and further reading

- Split Treatment: A Measurement of Coordination Between Psychiatrists and Nonmedical Therapists (audit of coordination; PubMed Central, PMC3004729).
- Sederer, Ellison and Keyes, "Guidelines for Prescribing Psychiatrists in Consultative, Collaborative, and Supervisory Relationships," *Psychiatric Services* 49(9), 1998.
- American Psychiatric Association, Commentary on Ethics in Practice (psychiatry.org).
- PRMS risk management guidance on split treatment (RM 0103).

ABOUT THIS DOCUMENT

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